

PACS Group (NYSE: PACS)

Skilled Nursing Care
United States



Company	PACS
Market Cap	\$5.8 B
FYE Revenue Growth	31.4%
Gross Margin	13.2%
EBITDA Margin	4.08%
Net Profit Margin	2.6%
ROE	23%
ROA	3%
Debt to Equity Ratio	4.1x
P/E Ratio - Current	34.7x
P/E Ratio – 5YR Avg.	N/A
P/E Ratio - Forward	18x

Company Overview

PACS Group is a healthcare services company that operates and manages skilled nursing facilities (SNFs) across the United States, focusing on post-acute care and rehabilitation services.

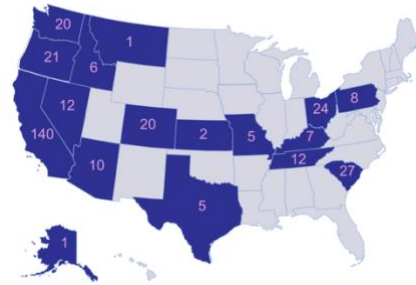
Its operations center on providing short-term rehabilitation, long-term care, and clinical services—including therapy and nursing care—primarily reimbursed through Medicare, Medicaid, and managed care.

Its growth strategy is acquisition-driven, targeting underperforming facilities and improving occupancy, payor mix, and operating efficiency.

Revenue by Geography: Their revenue stream primarily comes from California, Texas, and Arizona, with California

specifically contributing a majority of the company's revenue as of Dec 31st, 2025. Below is a graphic that shows the breakdown of the geographic diversification of their 321 facilities.

PACS at a Glance⁽¹⁾



Revenue Mix: Revenue mix is split among 4 main drivers: Medicare (40%), Medicaid (35%), Managed Care (15%), and Private (9%). With Medicare and Managed Care being the highest reimbursement rates and making up the “*Skilled Mix*” revenue portion. A breakdown of this reimbursement rate is below. This rate is set by the CMS Rating Agency.

	2025
Medicare	\$ 988.46
Managed care	688.43
Total for skilled patient payors ⁽¹⁾	881.22
Medicaid	322.91
Private and other	497.45
Total ⁽²⁾	\$ 525.57

Investment Thesis

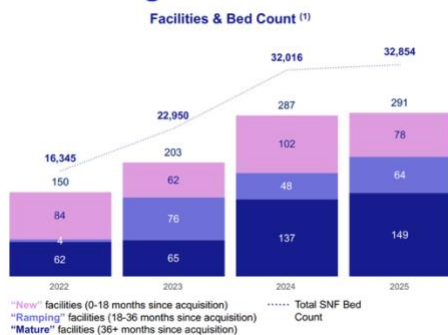
We rate PACS a **Buy with a 5Y Target Price of \$57 (11% CAGR)** per share driven by the following thesis points:

1) Most Successful Business Model Among Competitors

PACS Group represents one of the most effective operating models in the skilled nursing facility (SNF) industry, demonstrated not just by its acquisition strategy but by its **consistent, measurable improvements in clinical quality and facility performance post-acquisition**. The

company has grown its base of 4- and 5-star facilities by **~4.2x since 2020**, with **73%+ of facilities now rated 4 or 5 stars**, highlighting its ability to systematically upgrade underperforming assets. Unlike peers that rely on stable but stagnant portfolios, PACS executes a repeatable “acquire-and-optimize” strategy—driving rapid improvements through standardized clinical protocols, operational discipline, and centralized support infrastructure. This translates into both **higher-quality care and superior financial outcomes**, reinforcing a model that scales efficiently across geographies.

Maturing Cohorts of SNFs



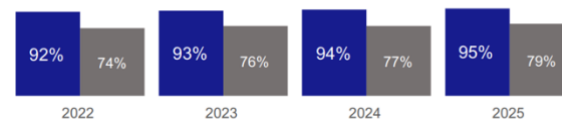
tangible performance advantages versus the industry, particularly in quality ratings and occupancy. Mature PACS facilities consistently achieve **~4.3–4.4 CMS star ratings vs. ~3.4–3.7 industry average**, alongside **occupancy rates of 92–95% vs. 74–79% for peers**. This gap is critical: higher ratings enhance referral flows from hospitals and payors, while higher occupancy directly drives revenue density and operating leverage. In effect, PACS is not just improving facilities—it is repositioning them as **preferred providers**

PACS Mature Facility QM Star Rating vs. Industry Average



in their local markets, creating a self-reinforcing cycle of volume growth and clinical reputation., creating a self-reinforcing cycle of volume growth and clinical reputation.

PACS Mature Facility Occupancy Rate vs. Industry Average



Importantly, PACS complements these operational gains with a focus on **payor mix and patient acuity**, prioritizing higher-reimbursement Medicare and managed care patients. This “skilled mix” strategy allows the company to increase **revenue per patient day and EBITDA margins** without relying solely on capacity expansion. When combined with industry-leading occupancy and quality scores, PACS is able to extract significantly more economic value per facility than peers that remain more heavily exposed to lower-margin Medicaid populations.



33.4%

Avg. Skilled Mix for Mature Facilities

2) Market Underestimating Revenue Growth and Margin Expansion

Significant Potential for Consolidation



Top-10 Providers⁽¹⁾



The market is underestimating PACS’ forward revenue growth, as it is anticipating a linear view of facility additions rather than the **exponential earnings power of maturing cohorts and operational efficiency**. While management has guided to roughly **5 facilities per quarter**, PACS has demonstrated an ability to scale far more aggressively when opportunities arise (**58 acquisitions in 2023 and 106 in 2024**), while still delivering strong execution. More importantly, revenue growth is not simply a function of new beds—PACS has already delivered **~46% revenue CAGR from 2021–2025 (\$1.2B to \$5.3B)**, driven by both acquisitions and post-acquisition efficiency. Given the highly fragmented market, PACS remains early in a long consolidation runway, and the market is likely underappreciating both the **pace and durability of this growth engine**.

Historical Performance

Twelve months ended December 31, 2025

PACS



At the same time, the market is materially underestimating **margin expansion driven by cohort maturation and improving unit economics**. As facilities transition from “new” to “mature,” PACS consistently drives step-function improvements: occupancy increases, average daily revenue, and skilled mix expand. These dynamics create a powerful embedded earnings lever, where even without incremental acquisitions, the existing portfolio continues to **compound EBITDA through operational improvement**. This is reinforced by PACS’ flywheel—better

clinical outcomes → higher ratings → stronger referral flows → higher occupancy and pricing—which suggests that recently acquired cohorts will follow the same trajectory. As a result, consensus is likely underestimating both **top-line growth and margin expansion**, as it fails to fully incorporate the magnitude and consistency of PACS’ post-acquisition value creation.



3) Management Will Effectively Implement Controls and Regain Compliance

The market is over-penalizing PACS for regulatory and legal overhangs that are **transitory, not structural**. Issues of this nature are common in the SNF industry and are typically resolved through **civil settlements and enhanced compliance**, rather than materially impairing operations. PACS’ centralized compliance infrastructure and scaled platform position it well to **remediate quickly and return to full alignment with regulatory standards**.

Importantly, core fundamentals remain intact—PACS continues to deliver **strong clinical outcomes, high CMS ratings, and industry-leading occupancy**, supporting stable referral flows and earnings power. As these issues are resolved, the removal of uncertainty should drive **multiple re-rating**, with investors refocusing on the company’s growth and margin expansion trajectory.

Competitive Positioning

Market

PACS' competitive edge stems from its ability to consistently outperform peers on the metrics that actually drive demand and profitability—clinical quality, occupancy, and payor mix—while scaling rapidly in a fragmented market. In an industry where ~89% of facilities are operated by smaller players, PACS combines a repeatable acquisition engine with a proven post-acquisition playbook that drives higher CMS ratings (~4.3–4.4 vs. ~3.4–3.7 industry) and superior occupancy (~92–95% vs. ~74–79%), positioning its facilities as preferred providers in local markets. This creates a structural advantage: better outcomes lead to stronger hospital referral relationships, which drive higher-acuity patients and improved reimbursement, reinforcing margin expansion.

Management

PACS' leadership team is led by co-founders Jason Murray and Mark Hancock, who bring over two decades of experience in post-acute healthcare and have been directly involved in operating skilled nursing facilities prior to founding the company. Both executives are licensed nursing home administrators, which is a meaningful differentiator—management is not purely financial, but deeply operational and clinically informed, enabling PACS to execute its facility turnaround strategy with precision. Josh Jergensen, President and COO, further reinforces this operator-first culture, with hands-on experience running complex SNFs and overseeing portfolio-wide operations.

This combination of founder-led vision, operator expertise, and growing institutional depth has enabled PACS to consistently acquire, integrate, and improve facilities at

scale, reinforcing its position as a top consolidator in the fragmented SNF market.

Moat

While skilled nursing facility operations are inherently local and fragmented, PACS has built a durable moat through its ability to systematically improve facility performance in ways that are difficult to replicate at scale. Although acquiring SNFs is accessible to many operators, consistently driving higher CMS ratings, occupancy (~92–95% vs. ~74–79% industry), and stronger payor mix requires a combination of clinical expertise, operational discipline, and standardized processes that most competitors lack. PACS' integration into hospital referral networks—driven by superior outcomes and reputation—creates embedded relationships and switching costs, as discharge planners prioritize high-performing facilities with proven care quality.

Additionally, PACS' enables local operators to remain agile while leveraging centralized systems, data, and best practices, reinforcing consistent execution across a growing portfolio. Its ability to repeatedly acquire underperforming assets and convert them into high-quality, high-occupancy facilities creates a self-reinforcing flywheel that improves both clinical outcomes and financial performance over time. Combined with its growing scale in a highly fragmented market, this positions PACS with a replicable yet difficult-to-match operating advantage, providing competitive insulation despite the commoditized nature of SNF assets.

Margin of Safety

PACS' margin of safety is supported by its proven ability to improve facilities after acquisition and drive consistent performance gains over time. Even if growth slows, the company can continue to increase earnings

by improving existing facilities, as shown by higher occupancy, better CMS ratings, and increased revenue per patient in mature locations. This means PACS does not rely only on new acquisitions to grow—it also benefits from built-in improvement within its current portfolio.

In addition, PACS operates in a highly fragmented and stable-demand industry, with strong long-term need for skilled nursing care. Its focus on higher-quality care and better outcomes helps maintain strong referral relationships and occupancy, even in tougher environments. Together, this combination of internal growth, steady demand, and operational consistency provides a buffer against downside risk and supports long-term performance.

Risks

Regulatory & Litigation Exposure:

PACS faces ongoing legal and regulatory scrutiny related to its billing practices and referral relationships, including Department of Justice investigations under the False Claims Act. These matters have led to internal reviews, financial restatements, and delayed filings, creating uncertainty around reported results and potential liabilities.

While the company is cooperating and strengthening compliance, these issues could result in fines, settlements, or operational changes, and continue to weigh on investor sentiment. Given the regulatory sensitivity of the SNF industry, adverse outcomes could impact margins and near-term growth expectations.

State Concentration Risk:

PACS has significant geographic concentration, with

more than half of its revenue derived from California, exposing the company to state-specific regulatory and reimbursement risk. California has one of the most complex and tightly regulated healthcare environments in the U.S., with frequent changes to Medicaid (Medi-Cal) reimbursement rates, staffing requirements, and compliance standards. Any unfavorable policy changes—such as rate cuts, increased labor mandates, or stricter enforcement—could disproportionately impact PACS’ financial performance.

Additionally, California tends to have higher labor costs and regulatory burden compared to other states, which can pressure margins if not offset by reimbursement increases. This concentration creates a risk that PACS’ overall results could be materially affected by changes in a single state, reducing diversification and increasing exposure to localized policy shifts. The company is actively investing in new states to mitigate this risk.

Stock-Based Compensation Dilution:

PACS also faces dilution risk through ongoing stock-based compensation issued to facility administrators and leadership, which is a key part of its incentive structure. While this aligns management with performance and supports retention of high-quality operators, it increases the share count over time and can dilute existing shareholders’ ownership and earnings per share. As PACS continues to grow and scale its platform, continued equity grants may be necessary to incentivize local operators, meaning that a portion of the company’s value creation is shared with employees. If not offset by sufficient earnings growth, this could limit per-share upside for investors despite strong underlying business performance.

Valuation

We applied five different methods to derive a 5-year target price of \$57 for PACS Group. These methods include DCF (25%), TEV/EBITDA (25%), and P/E (50%).

For the DCF method, we built out a revenue model based on several assumptions, such as continued occupancy levels, acquisitions of about 20 facilities per year, and a steady

Table – Management Profile

Jason Murray – President and Chief Executive Officer

Jason Murray is a Co-Founder and Chief Executive Officer of PACS Group Inc. and a member of the Board of Directors. Mr. Murray co-founded the company in 2012, focusing on scaling and shaping its acquisition-driven growth strategy and operating a portfolio of skilled nursing facilities. Prior to founding PACS, Mr. Murray worked in the post-acute care industry, including direct experience operating and managing skilled nursing facilities, which has informed PACS' emphasis on clinical quality and operational execution. Mr. Murray is a licensed nursing home administrator and brings over two decades of experience in healthcare operations, with a focus on scaling facility performance and integrating acquisitions across multiple markets.

Enterprise Value	10410.15728
Net Debt:	\$ 3,245.17
Equity Value	\$ 7,164.99
# Shares	156.62
Target Price (1Y)	\$ 45.75
Target Price (5Y)	\$ 56.89
Current Price:	\$ 33.44
Upside (5Y)	70%
CAGR (5Y)	11%

increase in CMS rates. Our operating margin assumptions were more aggressive than consensus, driven by the higher-than-expected graduation of facilities into the Mature Facility bucket.

Mark Hancock – President and Interim Chief Financial Officer

Mark is the Executive Vice Chairman, interim Chief Financial Officer and one of two founders/owners of PACS Group Inc., a rapidly growing national platform investing in the continuum of post-acute care, including post-acute care professionals, ancillary services, and 323 post-acute facilities in 17 states across the country. In addition to over 20 years of experience as a finance professional, Mark has previously worked as a licensed nursing home administrator in Utah, California, and Kentucky.

Prior to starting PACS, Mark worked for Fortune 500 companies in the auto and steel industries and as the treasurer for a \$20 billion financial services company. Mark was named a 2023 Mountain West Entrepreneur of the Year by Ernst & Young and was a finalist for their national Entrepreneur of the Year competition that same year. He is a Certified Treasury Professional (CTP) and holds a bachelor's degree in engineering from Brigham Young University, as well as a master's in business administration from Brigham Young University's Marriott School of Management.

Josh Jergensen – President, Chief Operations Officer

Josh Jergensen is President and Chief Operating Officer of PACS Group. Mr. Jergensen oversees day-to-day operations across the company's portfolio of skilled nursing facilities, with a focus on driving clinical quality, occupancy, and financial performance. Prior to his current role, he held various operational leadership positions within PACS and has extensive hands-on experience

managing and scaling skilled nursing facilities. His background as an operator reinforces PACS' emphasis on execution at the facility level, supporting the company's ability to integrate acquisitions and consistently improve performance across its platform.

John Mitchell – Chief Legal Officer & Secretary

As our Chief Legal Officer and Secretary, John manages the company's legal department. He earned his bachelor's degree and juris doctorate, magna cum laude, from Brigham Young University. John served as Vice President, Legal at HCP, a NYSE-traded healthcare-focused real estate investment trust, and as Senior Vice President, Legal, and Chief Compliance Officer, at Skilled Healthcare Group, a NYSE-traded skilled nursing company, prior to joining us in 2017. He spent the first portion of his career at an international law firm with a practice focus on corporate, finance, mergers, and acquisitions.

Michelle Lewis – Chief Accounting Officer

Michelle has served as our Executive Vice President and Chief Accounting Officer since January 2023. Since joining our company in July 2018, Ms. Lewis has served in various roles of increasing responsibility, including as our Controller. Prior to that, Ms. Lewis, who is licensed as a Certified Public Accountant, owned and operated Michelle Lewis Accounting Services, PLLC, a certified public accounting firm, and also provided controller functions at a privately held healthcare organization from January 2008 to May 2015. Ms. Lewis received a B.S. in Business Administration from the California State University East Bay. She has worked in the skilled nursing industry in various functions since 1997.

Trent Bingham – Chief Human Resource Officer

Trent joined PACS in November 2025, bringing more than two decades of human resources and management experience to the organization.

Throughout his career, Trent has held HR leadership roles across the healthcare industry—including pharmaceuticals (Eli Lilly & Company), medical devices (Edwards Lifesciences), and medical diagnostics (Renalytix), and began his career in patient care as a certified nursing assistant. His expertise spans HR strategy, Six Sigma, public accounting, and global business leadership, complemented by six years living and working abroad in Japan and Ireland.

Kelly Priegnitz – Chief Compliance Officer

Kelly Priegnitz joined PACS as Chief Compliance Officer in December 2025, the latest step in a decorated compliance and legal career.

She joined PACS after five years at Pinnacle Treatment Centers, where she served as Executive Vice President and Chief Compliance Officer. Before joining Pinnacle, Kelly served in legal and compliance leadership positions with Trilogy Health Services, Healogics, Kindred Healthcare,

and Sun Healthcare, roles in which she oversaw compliance and risk management within multiple areas of healthcare.

Key Financials

PACS GROUP, INC. AND SUBSIDIARIES
COMBINED/CONSOLIDATED BALANCE SHEETS
(dollars in thousands, except for share and per share values)

	December 31,	
	2025	2024
<u>ASSETS</u>		
Current Assets:		
Cash and cash equivalents	\$ 197,016	\$ 157,674
Accounts receivable, net	628,128	641,775
Other receivables	73,965	74,746
Prepaid expenses and other current assets	170,630	64,066
Total Current Assets	1,069,739	938,261
Property and equipment, net	1,201,096	990,580
Operating lease right-of-use assets	2,968,176	2,994,519
Insurance subsidiary deposits and investments	87,192	66,258
Escrow funds	18,404	25,122
Goodwill and other indefinite-lived assets	68,061	67,061
Other assets	171,366	161,108
Total Assets	\$ 5,584,034	\$ 5,242,909
<u>LIABILITIES AND EQUITY</u>		
Current Liabilities:		
Accounts payable	\$ 192,232	\$ 175,062
Accrued payroll and benefits	187,516	146,177
Current operating lease liabilities	153,066	136,232
Current maturities of long-term debt	4,463	14,852
Current portion of accrued self-insurance liabilities	128,994	75,966
Current line of credit	—	142,000
Refund liability	181,129	145,795
Other accrued expenses	154,030	142,348
Total Current Liabilities	1,001,430	978,432
Long-term operating lease liabilities	2,939,854	2,935,773
Line of credit	100,000	—
Long-term debt, less current maturities, net of deferred financing fees	244,803	250,984
Accrued self-insurance liabilities, less current portion	192,561	164,979
Other liabilities	152,937	197,050
Total Liabilities	\$ 4,631,585	\$ 4,527,218
Commitments and contingencies (Note 17)		
Equity:		
PACS Group, Inc. stockholders' equity:		
Common stock: \$0.001 par value; 1,250,000,000 shares authorized; 156,615,144 shares issued and outstanding as of December 31, 2025, and 155,177,511 shares issued and outstanding as of December 31, 2024	157	155
Additional paid-in capital	637,035	591,363
Retained earnings	309,579	118,036
Total PACS Group, Inc. stockholders' equity	946,771	709,554
Noncontrolling interest in subsidiary	5,678	6,137
Total Equity	\$ 952,449	\$ 715,691
Total Liabilities and Equity	\$ 5,584,034	\$ 5,242,909

PACS GROUP, INC. AND SUBSIDIARIES
COMBINED/CONSOLIDATED STATEMENTS OF INCOME
(dollars in thousands, except for share and per share values)

	Year Ended December 31,		
	2025	2024	2023
Revenue			
Patient and resident service revenue	\$ 5,287,885	\$ 4,086,655	\$ 3,110,114
Additional funding	—	—	375
Other revenues	1,047	3,079	1,003
Total Revenue	\$ 5,288,932	\$ 4,089,734	\$ 3,111,492
Operating Expenses			
Cost of services	4,129,696	3,297,091	2,447,713
Rent - cost of services	378,908	284,953	216,711
General and administrative expense	415,070	343,808	213,664
Depreciation and amortization	55,663	40,809	25,632
Total Operating Expenses	\$ 4,979,337	\$ 3,966,661	\$ 2,903,720
Operating income	309,595	123,073	207,772
Other (Expense) Income			
Interest expense	(28,363)	(44,341)	(49,919)
Gain on lease termination	—	8,046	—
Other income (expense), net	3,218	14,776	(536)
Total Other Expense, Net	\$ (25,145)	\$ (21,519)	\$ (50,455)
Income before provision for income taxes	284,450	101,554	157,317
Provision for income taxes	92,989	46,210	44,435
Net Income	\$ 191,461	\$ 55,344	\$ 112,882
Less:			
Net (loss) income attributable to noncontrolling interest	(82)	(416)	8
Net Income Attributable To PACS Group, Inc.	\$ 191,543	\$ 55,760	\$ 112,874
Net Income Per Share Attributable To PACS Group, Inc.			
Basic	\$ 1.23	\$ 0.38	\$ 0.88
Diluted	\$ 1.22	\$ 0.38	\$ 0.88
Weighted-Average Common Shares Outstanding			
Basic	156,180,786	146,663,371	128,723,386
Diluted	156,700,339	148,574,606	128,723,386

See accompanying notes to combined/consolidated financial statements.

PACS GROUP, INC. AND SUBSIDIARIES
COMBINED/CONSOLIDATED STATEMENTS OF STOCKHOLDERS' EQUITY
(dollars in thousands, except for share values)

	Common Stock		Additional Paid-in Capital	Retained Earnings	Noncontrolling Interest	Total
	Shares	Amount				
Balance January 1, 2023	128,723,386	\$ 129	\$ —	\$ 63,517	\$ 5,005	\$ 68,651
Contributions	—	—	—	—	587	587
Dividends on common stock (\$0.6246 per share)	—	—	—	(80,394)	—	(80,394)
Net income attributable to noncontrolling interest	—	—	—	—	8	8
Net income attributable to PACS Group, Inc.	—	—	—	112,874	—	112,874
Balance December 31, 2023	128,723,386	\$ 129	\$ —	\$ 95,997	\$ 5,600	\$ 101,726
Contributions	—	—	—	—	953	953
Issuance of common stock	24,206,350	24	509,418	—	—	509,442
Employee stock-based compensation	3,847,652	4	115,540	—	—	115,544
Tax withholdings related to net share settlement of equity awards	(1,599,877)	(2)	(33,595)	—	—	(33,597)
Dividends on common stock (\$0.2173 per share)	—	—	—	(33,721)	—	(33,721)
Net loss attributable to noncontrolling interest	—	—	—	—	(416)	(416)
Net income attributable to PACS Group, Inc.	—	—	—	55,760	—	55,760
Balance December 31, 2024	155,177,511	\$ 155	\$ 591,363	\$ 118,036	\$ 6,137	\$ 715,691
Employee stock-based compensation	—	—	54,069	—	—	54,069
Issuance of common stock to employees and directors resulting from the vesting of restricted stock unit awards	2,325,126	3	(3)	—	—	—
Tax withholdings related to net share settlement of equity awards	(887,493)	(1)	(8,394)	—	—	(8,395)
Noncontrolling interest distribution	—	—	—	—	(377)	(377)
Net loss attributable to noncontrolling interest	—	—	—	—	(82)	(82)
Net income attributable to PACS Group, Inc.	—	—	—	191,543	—	191,543
Balance December 31, 2025	156,615,144	\$ 157	\$ 637,035	\$ 309,579	\$ 5,678	\$ 952,449

See accompanying notes to combined/consolidated financial statements.

PACS GROUP, INC. AND SUBSIDIARIES
COMBINED/CONSOLIDATED STATEMENTS OF CASH FLOWS
(dollars in thousands)

	Year Ended December 31,		
	2025	2024	2023
Cash flows from operating activities			
Net income	\$ 191,461	\$ 55,344	\$ 112,882
Adjustments to reconcile net income to net cash provided by operating activities			
Depreciation and amortization	55,663	40,809	25,632
Amortization and write-off of deferred financing fees	3,148	3,115	6,068
Stock-based compensation	54,069	115,544	—
Loss on investment in partnership	773	4,565	391
Gain on insurance subsidiary deposits and investments	(4,896)	(1,258)	—
Deferred taxes	(17,733)	(44,484)	(9,923)
Noncash lease expense	48,454	34,647	22,727
Other noncash operating activities	573	843	581
Change in operating assets and liabilities			
Accounts receivable, net	13,647	(93,968)	(176,883)
Other receivables	781	(22,487)	(3,059)
Prepaid expenses and other current assets	(69,523)	(17,210)	(18,765)
Other assets	(2,660)	(13,338)	(3,338)
Escrow funds	6,718	(9,473)	568
Operating lease liabilities	(1,196)	(20,784)	(2,011)
Accounts payable	14,856	33,398	33,371
Accrued payroll and benefits	40,992	52,041	6,325
Accrued self-insurance liabilities	80,610	67,242	35,576
Refund liability	35,334	145,795	—
Other accrued expenses	13,227	73,210	(55,501)
Other liabilities	(60,074)	(36,210)	89,056
NET CASH PROVIDED BY OPERATING ACTIVITIES	\$ 404,224	\$ 367,341	\$ 63,697
Cash flows from investing activities			
Investment in partnerships	\$ —	\$ (32,891)	\$ (2,597)
Non-operating distributions from investment in partnership	5,593	3,521	1,862
Purchase of available-for-sale securities	(21,212)	(90,874)	—
Sale of available-for-sale securities	—	25,874	—
Acquisition of facilities	(143,751)	(283,344)	(127,024)
Purchase of property and equipment	(105,405)	(66,465)	(45,782)
Proceeds from the sale of assets	750	1,500	750
NET CASH USED IN INVESTING ACTIVITIES	\$ (264,025)	\$ (442,679)	\$ (172,791)
Cash flows from financing activities			
Borrowing on lines of credit, net of deferred financing fees	\$ —	\$ 676,000	\$ 855,704
Payments on lines of credit	(42,000)	(1,054,000)	(497,986)
Borrowings of long-term debt, net of deferred financing fees	—	72,305	411,313
Payments on long-term debt	(18,218)	(19,455)	(559,632)
Noncontrolling interest distribution	(377)	—	—
Contributions from noncontrolling interest	—	502	587
Dividends on common stock	—	(33,721)	(80,394)
Proceeds from initial public offering, net of issuance costs	—	414,157	—
Proceeds from common stock offering, net of issuance costs	—	95,285	—
Taxes paid related to net share settlement of equity awards	(8,395)	(33,597)	—
NET CASH (USED IN)/PROVIDED BY FINANCING ACTIVITIES	\$ (68,990)	\$ 117,476	\$ 129,592
Net change in cash	71,209	42,138	20,498
Cash, cash equivalents, and restricted cash - beginning of period	160,842	118,704	98,206
Cash, cash equivalents, and restricted cash - end of period	<u>\$ 232,051</u>	<u>\$ 160,842</u>	<u>\$ 118,704</u>
Cash and cash equivalents	\$ 197,016	\$ 157,674	\$ 73,416
Restricted cash (included in prepaid expenses and other current assets)	35,035	3,168	4,977
Restricted cash (included in other assets)	—	—	40,311
Cash, cash equivalents, and restricted cash	<u>\$ 232,051</u>	<u>\$ 160,842</u>	<u>\$ 118,704</u>

Competitor Data

Ensign Group Facility Breakdown

	Operated Facilities									
	Leased without a Purchase Option		Leased with a Purchase Option		Owned		Total			
	Facilities	Beds/Units	Facilities	Beds/Units	Facilities	Beds/Units	Facilities	Beds/Units	Facilities	Beds/Units
Texas	56	6,975	2	220	27	3,632			85	10,847
California	73	7,243	—	—	12	1,333			85	8,576
Arizona	24	3,515	—	—	17	2,546			41	6,061
Colorado	29	3,088	1	125	9	961			39	4,174
Utah	12	1,314	2	159	15	1,102			29	2,575
Washington	12	1,085	—	—	6	621			18	1,706
Idaho	9	732	—	—	6	590			15	1,322
Kansas	2	147	3	325	7	662			12	1,134
Tennessee	6	822	—	—	3	300			11	1,122
South Carolina	4	582	—	—	5	544			9	1,126
Iowa	6	399	—	—	3	234			9	633
Nebraska	5	364	—	—	3	331			8	695
Wisconsin	—	—	—	—	4	302			4	302
Nevada	3	483	—	—	—	—			3	483
Alaska	—	—	—	—	2	228			2	228
Alabama	2	181	—	—	—	—			2	181
Oregon	—	—	—	—	1	148			1	148
	245	26,930	8	829	120	13,554			373	41,313

	Name (BICS Best Fit)	Exp Report Date	BEST Sales (Cur Qtr)	BEST EBITDA (Cur Qtr)	BEST P/E (Cur Qtr)	BEST EPS Mean (Cur Qtr)	BEST EPS Low (Cur Qtr)	BEST EPS High (Cur Qtr)	Number of Ests (Cur Qtr)	BEST EPS 1mo % Chg (Cur	BEST EPS 3mo % Chg (Cur
	Median		553.43M	64.30M	17.40	0.31	0.22	0.35	9.00	0.00%	2.44%
01	PACS GROUP INC	05/13/26	1.36B	113.80M	15.06	0.42	0.41	0.43	4.00	0.48%	2.44%
02	ALIGNMENT HEALTHCARE INC	05/01/26	1.22B	32.31M	99.82	0.03	-0.01	0.10	11.00	0.00%	3.23%
03	RADNET INC	05/11/26	562.20M	61.76M	90.82	-0.18	-0.33	-0.02	5.00	0.00%	-348.78%
04	SOTERA HEALTH CO	05/01/26	270.70M	131.40M	15.22	0.17	0.15	0.18	10.00	0.00%	-1.69%
05	HEALTH EQUITY INC	06/03/26	352.13M	152.53M	17.40	1.12	1.05	1.26	15.00	-1.32%	-2.09%
06	CONCENTRA GROUP HOLDINGS PAR	05/07/26	553.43M	109.14M	14.84	0.33	0.32	0.35	7.00	-0.89%	-7.99%
07	LIFESTANCE HEALTH GROUP INC	05/07/26	387.56M	42.50M	43.90	0.03	0.00	0.06	9.00	0.00%	56.25%
08	PEDIATRIX MEDICAL GROUP INC	05/06/26	465.38M	51.98M	9.85	0.38	0.36	0.41	7.00	0.00%	4.14%
09	ADDUS HOMECARE CORP	05/05/26	366.57M	44.41M	13.52	1.55	1.41	1.61	14.00	-0.39%	-1.65%
10	BRIGHTSPRING HEALTH SERVICES	05/01/26	3.38B	166.93M	27.37	0.31	0.21	0.47	16.00	1.95%	14.18%
11	ASTRANA HEALTH INC	05/08/26	944.50M	64.30M	23.07	0.28	0.22	0.35	9.00	6.06%	4.87%

Bloomberg and CapIQ Graphs

